Nivolumab 3 mg/kg + ipilimumab 1 mg/kg — advanced melanoma

To help immune cells control advanced melanoma with two checkpoint medicines, followed when appropriate by nivolumab alone. The chosen dose combination has specific licensing and evidence limitations to discuss during consent.

Advanced melanoma · NCCP 00551 v7a · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

NCCP identifies nivolumab 3 mg/kg plus ipilimumab 1 mg/kg for melanoma as an unlicensed dosing posology in Ireland, despite reimbursement. Your clinician should explain this, the available alternatives and uncertainties, document informed consent under hospital policy, and follow the hospital Drug and Therapeutics Committee or equivalent acknowledgement process. CheckMate 511 primarily compared safety; it did not prove that this schedule has equal survival benefit to nivolumab 1 mg/kg plus ipilimumab 3 mg/kg.

My overall treatment

Up to four combination infusions, 21 days apart, then nivolumab alone every two or four weeks as prescribed; four stamps record combination doses only.

Combination	Nivolumab alone
Nivolumab 3 / ipilimumab 1 IV every 21 days, up to 4 doses. This melanoma dose combination is unlicensed in Ireland and needs specific consent.	Two-weekly maintenance starts 3 weeks after last combination; four-weekly starts 6 weeks after. Team confirms IV/SC route and dose; prior toxicity still applies.

A typical 21-day cycle

Day 1	Day 2–21
Treatment day	Between treatment visits

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline safety** NCCP / current product information / individual plan	Blood count, kidneys/liver, glucose, thyroid, BRAF and immune review; specific consent for unlicensed3/1 melanoma schedule.
During	**Safety checks** NCCP / current product information / individual plan	Same blood panel before every cycle; symptom-triggered tests. Confirm maintenance route, interval and first dose date.
After / response	**Follow-up** NCCP / current product information / individual plan	Monitor immune effects for at least five months after last dose; scans individually planned.

Between treatments

- Four stamps track combination only. New diarrhoea, breathlessness, severe headache, weakness or rash needs early contact, including after treatment.

Bring medicine list, test dates and actual oncology/out-of-hours numbers. Example numbers must be replaced.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / hormones

Immunotherapy

Severe headache, dizziness, profound tiredness, thirst or altered vision can signal pituitary, adrenal, thyroid or glucose problems. Symptoms may need urgent hormone tests and treatment.

3 Heart

Immunotherapy

Rare heart inflammation can cause chest pain, palpitations, faintness or breathlessness. Severe chest pain, collapse or major breathing difficulty is an emergency.

5 Liver

Immunotherapy

Liver inflammation may first appear in blood tests. Yellow eyes or skin, dark urine, severe nausea or right-sided abdominal pain also need prompt review.

7 Nerves / muscles

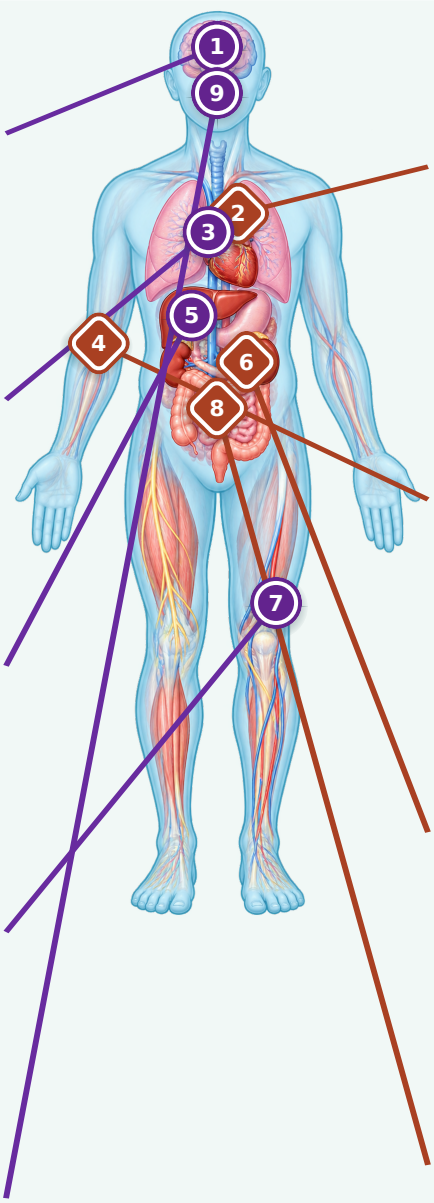
Immunotherapy

New weakness, drooping eyelids, double vision, numbness or difficulty walking, swallowing or breathing may be a rare nerve or muscle effect. Seek urgent help; do not wait for the next visit.

9 Eyes

Immunotherapy

Eye pain, redness, light sensitivity or changed vision can indicate inflammation. Get prompt assessment; do not assume it is ordinary tiredness.



2 Lungs

Immunotherapy

New cough, shortness of breath or chest discomfort can mean lung inflammation or infection. Report changes promptly, even with normal recent blood results.

4 Skin

Immunotherapy

Rash and itching can occur. Rapid spreading, blistering, peeling or sores involving the mouth, eyes or genitals require urgent assessment.

6 Kidneys / urine

Immunotherapy

Kidney inflammation may be detected in blood tests. Report reduced urine, blood in urine, swelling or a marked change in thirst.

8 Bowels

Immunotherapy

New diarrhoea, blood or mucus, severe cramps or abdominal pain can mean bowel inflammation. Record changes from your usual pattern and call early; severe pain or bleeding needs urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Nivolumab 3 + ipilimumab 1

Combination 3 mg/kg + 1 mg/kg every 21 days, up to 4 doses; then nivolumab alone as prescribed.

Phase: _____ Planned cycles: 4

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1 — Date / initials

C2 — Date / initials

C3 — Date / initials

C4 — Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.